Reese Spine & Injury Center

18188 Rebecca Keys Suite 231
Pearland, TX
Phone 832-555-3292

Itemized Medical Bill

Patient:	Gabriel Hernandez
Date of birth:	1988-08-22
Insurance claim:	C-1352
Date of injury:	2024-09-21
First visit:	2024-09-28
Last visit:	2025-04-06
Number of visits:	29

Services

Service	Units	Charge
99203 New patient evaluation	1	$321.28
98941 Spinal manipulation	3	$300.32
97110 Therapeutic exercise	2	$405.97
97124 Massage therapy	6	$593.92
72040 X-ray, cervical spine	8	$6,539.37
20552 Trigger point injection	9	$4,151.85
TOTAL		$12,312.71

Recent visits

Date	Service	Charge
2025-02-27	72040 X-ray, cervical spine	$572.49
2025-03-08	98941 Spinal manipulation	$107.60
2025-03-15	20552 Trigger point injection	$413.62
2025-03-24	20552 Trigger point injection	$499.74
2025-04-01	20552 Trigger point injection	$480.34
2025-04-06	20552 Trigger point injection	$485.53

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Treatment History and Summary

Patient:	Gabriel Hernandez
Treating provider:	Timothy York, DC
Diagnosis:	shoulder injury

Treatment history by month

Month and services	Visits	Charges
September 2024: evaluation	1	$321.28
October 2024: massage therapy, pain management, physical therapy	4	$914.95
November 2024: imaging, massage therapy, pain management	5	$2,452.30
December 2024: imaging, massage therapy, pain management	4	$1,205.13
January 2025: chiropractic adjustment, imaging, pain management	5	$3,000.06
February 2025: chiropractic adjustment, imaging, physical therapy	5	$2,432.16
March 2025: chiropractic adjustment, pain management	3	$1,020.96
April 2025: pain management	2	$965.87

Treatment summary

Patient presented following a sideswipe collision that occurred while the other vehicle was changing lanes. The reported mechanism is consistent with the shoulder complaints documented at intake, and the injury pattern is plausible for a impact of this type. Over the course of 29 visits, care followed a conventional progression, beginning with physical therapy, range-of-motion work, and home exercise instruction, then tapering in frequency as symptoms improved and strength returned. Physical examination revealed limited abduction and tenderness over the rotator cuff insertion, findings that support the clinical diagnosis, while several neurological screens and imaging-independent stability tests returned normal results. The patient reports no prior injury or treatment to this shoulder, and no earlier records addressing this body region were identified in the chart review. Billing reflects one charge per scheduled service, with no dulpicate or repeated entries. At discharge the patient had reached a stable functional level with residual mild stiffness, and continued home exercise was recommended.